

SOCIAL DETERMINANTS OF HEALTH

EXAMINING SYSTEMIC IMPACTS ON VULNERABLE POPULATIONS



CREATED FOR

BPM2 DM SG 01 HPWP 3

CREATED BY

SOM MULTICULTURAL CURRICULUM WORKING GROUP

TODAY'S LECTURER

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RESPECT FOR ALL PERSONS

"Wherever the art of medicine is loved, there is also a love of humanity."
– Hippocrates

Respect is a core value in medicine, ensuring dignity, fairness, and justice for all patients.

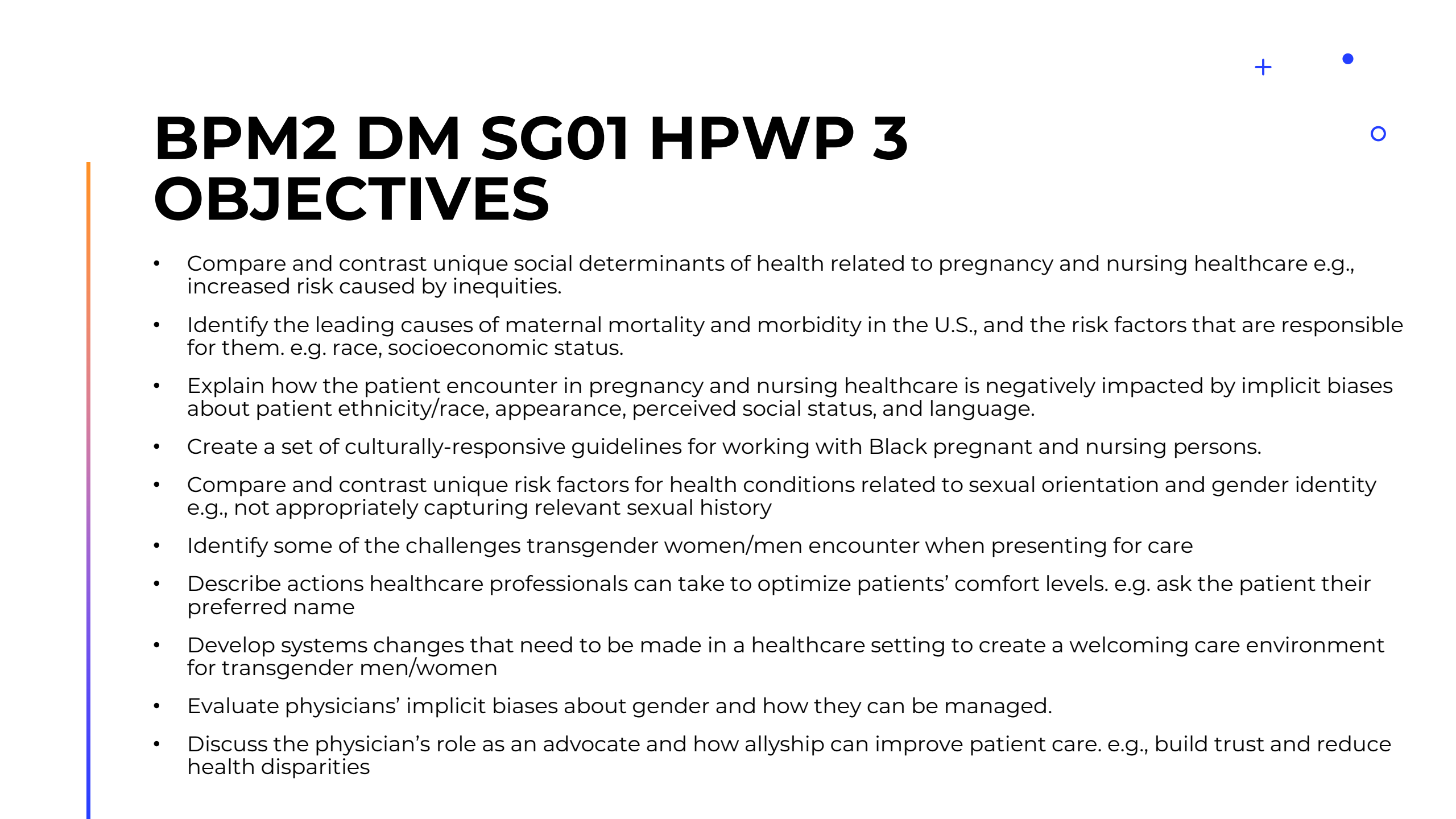
Respect fosters trust, improves communication, and enhances patient satisfaction and outcomes.

Demonstrating respect enhances the reputation and integrity of the medical profession.



SOM MULTICULTURAL CURRICULUM

- BPM1 Patients as teacher – A Multicultural Approach to Exploring Bias in Healthcare
- BPM2 Social Determinants of Health – Examining Systemic Impacts on Vulnerable Populations
- BPM3 Ethical Considerations in Public Health – Health Related Limitations and Informed Consent
- PCM1 Social Determinants of Health – Focus on the Patients, Not the Diagnosis
- PCM2 Ethical Considerations in Healthcare – Care Across Cultures, Class and Clinical Decision-Making



BPM2 DM SG01 HPWP 3

OBJECTIVES

- Compare and contrast unique social determinants of health related to pregnancy and nursing healthcare e.g., increased risk caused by inequities.
- Identify the leading causes of maternal mortality and morbidity in the U.S., and the risk factors that are responsible for them. e.g. race, socioeconomic status.
- Explain how the patient encounter in pregnancy and nursing healthcare is negatively impacted by implicit biases about patient ethnicity/race, appearance, perceived social status, and language.
- Create a set of culturally-responsive guidelines for working with Black pregnant and nursing persons.
- Compare and contrast unique risk factors for health conditions related to sexual orientation and gender identity e.g., not appropriately capturing relevant sexual history
- Identify some of the challenges transgender women/men encounter when presenting for care
- Describe actions healthcare professionals can take to optimize patients' comfort levels. e.g. ask the patient their preferred name
- Develop systems changes that need to be made in a healthcare setting to create a welcoming care environment for transgender men/women
- Evaluate physicians' implicit biases about gender and how they can be managed.
- Discuss the physician's role as an advocate and how allyship can improve patient care. e.g., build trust and reduce health disparities



SESSION PLANNING

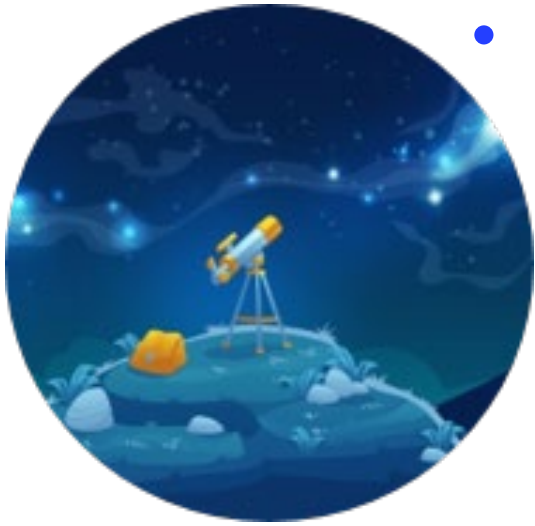
Discuss terminology

Discuss social determinants of health
and relation to health disparities

Health disparities for pregnant and
nursing people and LGBTQIA+ folks

Explore patient perspectives of
physician-patient encounters

Discuss standards of care



RESPECT THE SPACE

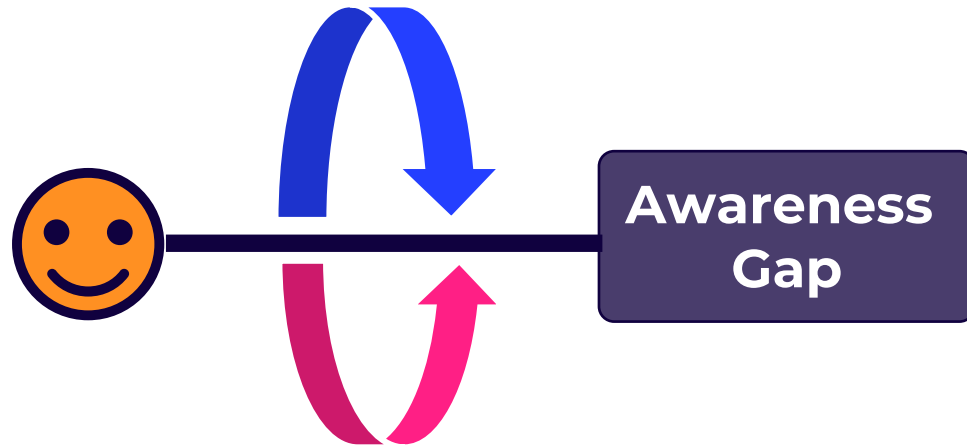
Show sensitivity towards each other's vulnerabilities and respect the space without judgement.

GROUND RULES FOR GROUP DISCUSSIONS

1. Listen actively.
2. Respectfully challenge one another, focus on ideas not the person.
3. Share your own story and experience, instead of invalidating somebody else's story.
4. The goal is not to agree – it is about hearing and exploring divergent perspectives.
5. Be conscious of body language and nonverbal responses.

INTENT VS. IMPACT

**We judge ourselves
by our intentions.**

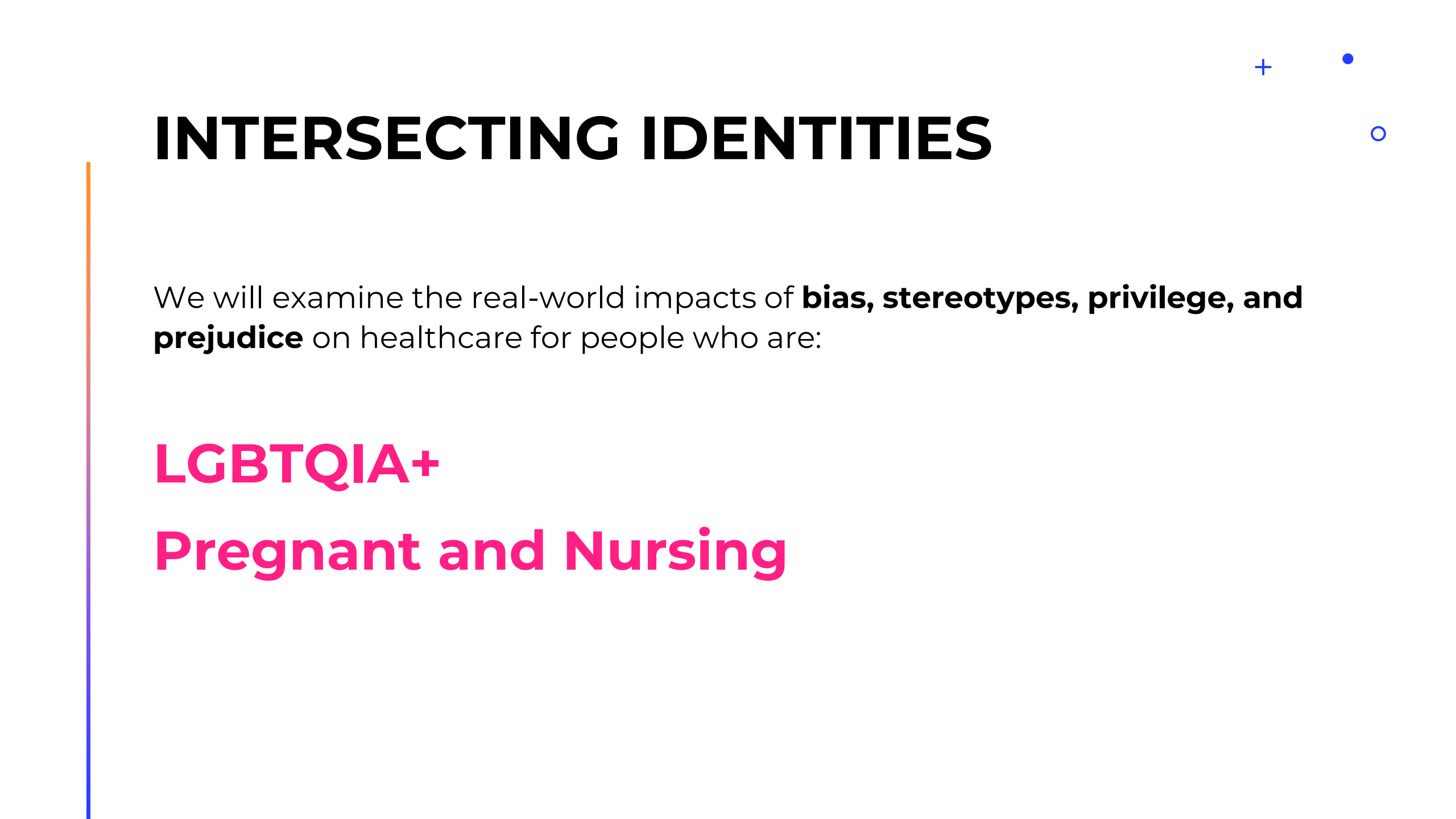


**We judge others
by their impact.**

As a physician, what matters more?

☐ **What you said (intent)?**

☐ **What your patient heard
(impact)?**



INTERSECTING IDENTITIES

We will examine the real-world impacts of **bias, stereotypes, privilege, and prejudice** on healthcare for people who are:

LGBTQIA+

Pregnant and Nursing



PRIVILEGE

Privilege refers to certain social advantages, prestige, and respect given to one person or group of people and not to others solely based on membership or perceived membership.

E.g., ability, education, class, religion, race, gender identity, language



Spend 30s thinking about this question.

**I CAN AFFORD TO VISIT A
HEALTHCARE PROFESSIONAL
IF NEEDED, MULTIPLE TIMES
PER YEAR.**

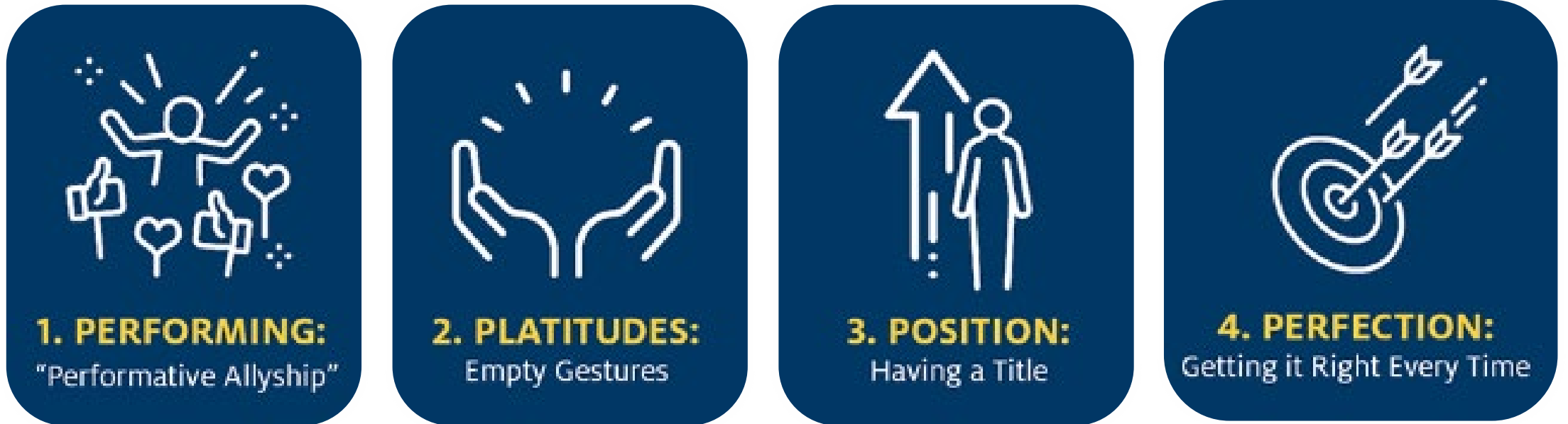
MICROAGGRESSIONS

Microaggression refers to “brief, commonplace, daily indignities...that communicate negative or derogatory slights.” Eg.

- “You speak English well.”
- “People aren’t really racist anymore; people are just more sensitive.”
- Assuming female doctor is a nurse/male nurse is an orderly.
- Catcalls

ALLYSHIP IS A SKILL ...

Here are 4 Common Misconceptions of Allyship:



... AND IT CAN BE PRACTICED!

MODIFIED FROM [WHAT IS ALLYSHIP? YOUR QUESTIONS, ANSWERED](#) | CCL AND AAMC MEDICAL EDUCATION GUIDE [AAMC.ORG/LGBTDSD](https://www.aamc.org/lgbtdsd)

SOCIAL DETERMINANTS OF HEALTH (SDOH)

Non-medical factors that influence health outcomes
Result of social and economic systems (WHO), 2022)

**Health and
Health Care**

**Economic
Stability**

Education

**Social and
Community Context**

**Neighborhood
Environment**

E.g. Access to medical insurance for healthcare financing



EMPATHY AND COMPASSION IN HEALTHCARE DELIVERY

Empathy is the ability to emotionally understand what your patients are feeling; See things from their perspectives; and imagine yourself in their place.

Practicing Compassion involves feeling another person's pain and taking steps to help relieve their suffering.

WHAT ARE BARRIERS AND PITFALLS?



Let's learn about

PREGNANT AND NURSING HEALTHCARE



PREGNANT AND NURSING HEALTHCARE

“Care by skilled health professionals before, during and after childbirth can save the lives of women and newborns”

WHO, 2024

“Every day in 2020, almost 800 women died from preventable causes related to pregnancy and childbirth.”

WHO, 2024



PREGNANT AND NURSING HEALTHCARE

Childbirth - natural process involving highly-complicated and -orchestrated biochemical, physical, emotional, and mental events

Some risks of pregnancy, labor, and delivery:

- ✓ gestational diabetes
- ✓ vaginal hematoma
- ✓ postpartum depression and anxiety
- ✓ life-threatening hypertension
- ✓ death

Increased risks caused by inequities in access to appropriate and timely

- ✓ prenatal and postnatal care,
- ✓ nutritional support
- ✓ health interventions

Moderating effect of the social determinants of health.

PREGNANCY RELATED COSTS

Women Who Give Birth Incur Nearly \$19,000 More in Health Costs Than Women Who Don't

Average additional health spending associated with pregnancy for women with large employer coverage, 2018-2020

Additional spending associated with pregnancy

\$18,865

Paid by insurance

\$16,011

Paid out-of-pocket

\$2,854

NOTES: Includes health spending throughout pregnancy, delivery, and postpartum care. For more, see "Health Costs Associated with Pregnancy, Childbirth, and Postpartum Care for People with Private Health Insurance."

Peterson-KFF
Health System Tracker

A TALE OF TWO BIRTHS IN CALIFORNIA

SARA and **MAYA**, each expecting her first child, have similar low-risk pregnancies. How different can their birth experiences be?

The experiences of pregnant mothers at California hospitals—small or large, urban or rural—can vary dramatically.



LOW-RISK C-SECTION

In low-risk pregnancies, C-sections should be avoided to reduce post-surgical complications.

Lower rate is better.



EPISIOTOMY

While this minor surgical procedure makes more space for the baby's birth, it may lead to complications for the mother.

Lower rate is better.



EXCLUSIVE BREASTFEEDING BEFORE DISCHARGE

Breastfeeding benefits mother and baby.

Higher rate is better.



VAGINAL BIRTH AFTER C-SECTION (VBAC)

Women who have had a C-section do not necessarily need to deliver subsequent babies by C-section.

Higher rate is better.



SARA

GOES TO A HIGH-PERFORMING HOSPITAL.

THE LIKELIHOOD THAT SHE WILL EXPERIENCE THESE PROCEDURES IS:

19%

2%

88%

27%



MAYA

GOES TO A LOW-PERFORMING HOSPITAL.

THE LIKELIHOOD THAT SHE WILL EXPERIENCE THESE PROCEDURES IS:

56%

46%

19%

1%

WHAT IF THIS VARIATION DID NOT EXIST?
If all California hospitals were high performing on maternity measures, it would mean (annually):

14,800
[31%]
FEWER C-SECTIONS

38,900
[84%]
FEWER EPISIOTOMIES

119,900
[41%]
MORE BABIES BREASTFED

12,300
[144%]
MORE VBACS

Learn about the performance of hospitals near you at www.CalHospitalCompare.org.

The California Hospital Assessment and Reporting Taskforce (CHART) used data provided by the California Maternal Quality Care Collaborative (CMQCC) for this project. CHART is a nonprofit established to develop a statewide hospital performance reporting system using a multi-stakeholder collaborative process. CMQCC seeks to improve maternal and infant outcomes through rapid-cycle data analytics and collaborative action. Percentages reflect the weighted average of all California hospitals in the high- and low-performing categories, respectively, for each maternity measure. Learn more about the data at www.chcf.org/2births. ©2014 California HealthCare Foundation.

Examining Disparities in Pregnant and Nursing Healthcare

- How does hospital infrastructure, location or health system contribute to disparities for those pregnant or nursing?
- What does this infographic reveal?

SDoH IN ACTION

THREE DELAYS MODEL



SDoH IN ACTION

THREE DELAYS MODEL



As reflected in this framework, delays to life-saving care occur at the individual, community, and facility levels.



Spend 30s thinking about this question.

**HOW FAR IS THE NEAREST
HOSPITAL FROM WHERE YOU
LIVED BEFORE MOVING
TO GRENADA?**

INDIGENOUS WOMEN AND PREGNANCY RELATED HEALTHCARE



PREGNANCY RELATED DEATHS AMONG INDIGENOUS WOMEN (USA)

Table 1. Underlying causes of pregnancy-related death among American Indian or Alaska Native persons, data from Maternal Mortality Review Committees in 36 US states, 2017–2019¹

	N	%
Mental health conditions ²	5	31.3
Hemorrhage ³	3	18.8
Amniotic fluid embolism	2	12.5
Infection	2	12.5
Cardiac and coronary conditions ⁴	1	6.3
Collagen vascular/autoimmune diseases	1	6.3
Conditions unique to pregnancy ⁵	1	6.3
Injury ⁶	1	6.3

Table 2. Distribution of pregnancy-related deaths among American Indian or Alaska Native persons by timing of death in relation to pregnancy, data from Maternal Mortality Review Committees in 36 US states, 2017–2019*

	n	%
During pregnancy	2	11.8
Day of delivery	3	17.7
1–6 days postpartum	1	5.9
7–42 days postpartum	6	35.3
43–365 days postpartum	5	29.4

UK PREGNANCY RELATED MORTALITY RATES

Maternal deaths by ethnicity England 2016-2018

Ethnic group	Total maternities	Deaths	Death rate per 100,000 maternities	Relative Risk: BAME compared with white		
				Relative Risk ratio	Lower CI	Upper CI
White	1,486,428	117	7.9	1.0		
Asian	191,145	28	14.7	1.9	1.2	2.8
Black	81,704	28	34.3	4.4	2.8	6.6
Chinese/other	75,270	6	8.0	1.0	0.4	2.3
Mixed race	31,823	8	25.1	3.2	1.4	6.5

Source: [Confidential Enquiry into Maternal Deaths 2016-18, Table 2.10](#)

Further details of MBBACE-UK's work is available on its [website](#).

BEST CLINICAL PRACTICES FOR THIS POPULATION



Empathy and compassion to foster patient trust

Use:

- ✓ Structured history-taking (including open-ended questions, active listening, and appropriate body language)
- ✓ Adequate review of patients' medical histories and risks to inform care plan

Consider practical applications of the aforementioned:

- ✓ Reviewing an electronic health record
- ✓ Embedding cultural competency into patient-clinician interactions
- ✓ Remaining cognizant of possible SDoH

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[Communicating with LGBTQ patients | American Medical Association \(ama-assn.org\)](#)

[Terminology and definitions | Gender Affirming Health Program \(ucsf.edu\)](#)